Educational information only. Not a substitute for medical advice.

Break the K Loop

A first-principles workbook to stop recreational ketamine use

In the voice of Miss K – your Ketaverse guide

Educational use only. Not medical advice.

Welcome, crew.

I’m Miss K. This workbook is here to help you stop using ketamine by breaking the loop that keeps pulling you back in. No judgement. No lectures. Just a clear system you can actually run when life gets loud.

How to use this workbook

- [ ] Work through sessions in order (2 per week is ideal).
- [ ] Do the homework - it is small, but it is the part that rewires the loop.
- [ ] Keep it simple. If a step feels too big, shrink it to a 10-minute version.
- [ ] If you slip, use the slip repair section and come back. Data, not shame.

Course promise: We do not rely on motivation. We change the inputs that make using likely: availability, proximity, cues, stories, and low-resistance moments.

What this is not: This is not medical advice or treatment. If you are unwell, unsafe, or have severe symptoms, seek professional help.

Safety and support

Ketamine is a dissociative anaesthetic. Recreational use can carry real risks, including accidents and significant urinary/bladder harm with repeated misuse. If you have bladder pain, blood in urine, severe frequency or urgency, or worsening symptoms, get checked by a clinician.

If someone is collapsed, unresponsive, having seizures, struggling to breathe, or severely confused, treat it as an emergency and call your local emergency number.

UK support routes (add your local options here):

- [ ] NHS: speak to your GP or local alcohol and drug service.
- [ ] FRANK (Talk to Frank): information and signposting.
- [ ] SMART Recovery: skills-based peer support meetings (online and local).

Miss K note: asking for support is not weakness. It is engineering.

The Miss K model: the five switches

Ketamine use usually needs five switches to line up. If we switch off even one, the loop weakens. Switch off three or more and the loop collapses.

1. Availability	How quickly can you get it? How easy is it right now?
2. Proximity	Are you around people/places where it is normal?
3. Cue	Stress, boredom, loneliness, nightlife, comedown, payday.
4. Story	"Just once", "I deserve it", "I need it to cope".
5. Low resistance	Tired, drunk, emotional, afters, late nights.

Miss K rule: Add friction to using. Remove friction from coping. Replace the job ketamine is doing.

6-week roadmap

Two sessions per week. Keep it simple. Do the homework. If you miss a week, you do not restart from zero - you continue from the next step.

Week 1

S1 Map your loop • S2 What job is ketamine doing?

Outcome: See the pattern and name the job to replace.

Week 2

S3 Add friction • S4 People, places & scripts

Outcome: Make using harder and handle pressure calmly.

Week 3

S5 10-minute Urge Protocol • S6 Story engineering

Outcome: Turn cravings into a routine; neutralise permission thoughts.

Week 4

S7 Track A/B days • S8 Minimum viable recovery

Outcome: Build high-risk day plans and reduce low-resistance states.

Week 5

S9 Night Out Protocol • S10 Slip repair (data, not shame)

Outcome: Stay ketamine-free on nights out; upgrade the system fast.

Week 6

S11 Support scaffolding • S12 Your 90-day manual

Outcome: Add support that lowers relapse risk and lock it in.

Session 1 - Map your loop

Objective: See your pattern clearly: cue, access, story, use, aftermath.

Miss K teaching:

- Miss K rule: if you can predict it, you can interrupt it.
- We are not fighting you. We are redesigning the system around you.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write your last 3 uses. What happened in the 2 hours before each one?
- [ ] Circle the first moment the chain could have been interrupted.

Homework: Complete the Trigger and Urge Log for 7 days (2 minutes per entry).

Session 2 - What job is ketamine doing?

Objective: Identify what ketamine is trying to solve so you can replace the function.

Miss K teaching:

• Common jobs: switch-off, numbness, anxiety quieting, confidence, enhancement, comedown management.

• If the job stays open, ketamine will keep getting rehired.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write: 'Ketamine helps me by...' and 'Ketamine costs me by...'.
- [ ] Choose your top 2 jobs to replace first.

Homework: Pick a 10-minute and a 60-minute replacement for each job and make them easy to start.

Session 3 - Add friction (access shutdown)

Objective: Make using meaningfully harder by switching off availability and proximity.

Miss K teaching:

- Behaviour follows friction. If using is the easiest option, it wins.
- Your goal is not perfection; your goal is fewer easy paths to K.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Choose 5 friction moves (minimum 3): block contacts, remove kit, remove stashes, avoid hotspots, money friction, change routes, pre-book travel, accountability check-ins.
- [ ] Set your 72-hour safeguards.

Homework: Implement your safeguards within 24 hours and note what changed.

Session 4 - People, places and scripts

Objective: Handle pressure and 'afters logic' without long explanations.

Miss K teaching:

- Most relapses start earlier than the drug - usually with a yes to the wrong room.
- Short scripts beat willpower.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Pick 3 boundary scripts and write them in your own words.
- [ ] Create an exit plan: leave time + transport + one person who knows.

Homework: Send one boundary message today. Short, calm, firm.

Session 5 - The 10-minute Urge Protocol

Objective: Turn cravings into a repeatable routine instead of a crisis.

Miss K teaching:

- Urges rise and fall. Access plus story is what converts an urge into use.
- We train your autopilot. Not your motivation.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Build your D-D-D-R card: Delay 10 minutes, Disrupt body state, Defuse the story, Replace with your job substitute.
- [ ] Choose your disrupt action (walk, cold water, stairs, squats).

Homework: Practice the protocol once per day, even with mild urges.

Session 6 - Story engineering

Objective: Neutralise the permission thoughts that reopen the door.

Miss K teaching:

- Your brain will negotiate: 'just once', 'I'll control it', 'I need it to sleep'.
- Counter-scripts must be short, believable and action-directing.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write counter-scripts for your top 3 relapse stories.
- [ ] Put them somewhere you will see them when tired.

Homework: Use your counter-script the moment you notice negotiating.

Session 7 - Track A / Track B days

Objective: Build two operating modes: normal days and high-risk days.

Miss K teaching:

• Relapse is often a state: tired, alone, stressed, bored, drinking.

• A high-risk day needs a different plan before the urge arrives.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Create Track A (normal) and Track B (high-risk).
- [ ] Track B must include food, movement, connection, and an off-ramp time.

Homework: Run Track B once on purpose this week.

Session 8 - Minimum viable recovery

Objective: Reduce low-resistance states with a 15-minute reset routine.

Miss K teaching:

• You do not need perfect self-care. You need minimum viable routines that run on bad days.

• Small stabilisers reduce cravings: sleep, food, hydration, light movement.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write your 15-minute routine (tidy, shower/air, water + food, breathing/stretch).
- [ ] Choose a time trigger (after work, after event, before bed).

Homework: Do the routine 4 times this week and track urge intensity before/after.

Session 9 - The Night Out Protocol

Objective: Stay ketamine-free in real nightlife settings without losing your life.

Miss K teaching:

- Nights out fail without pre-commitments: leave time, travel, buddy, alcohol rule.
- Miss K rule: leave before the room changes.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Fill your plan: arrival time, leave time, travel booked, check-ins, exit script.
- [ ] Set your 'leave now' warning sign (restlessness, bargaining, chasing).

Homework: Run one social event using the protocol (even a small one).

Session 10 - Slip repair (data, not shame)

Objective: Turn a slip into system upgrades within 24 hours.

Miss K teaching:

• A slip is a systems failure, not a character failure.

• Fix the inputs: sleep, friction, access, and support.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write your 24-hour reset checklist.
- [ ] Write the relapse chain: where did it start?

Homework: Share your slip plan with one supporter so you are not alone with it.

Session 11 - Support scaffolding

Objective: Build support that reduces relapse risk over time.

Miss K teaching:

• Hard change is easier with structure: check-ins, groups, professional help when needed.

• Support is engineering, not weakness.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Choose one support action: accountability partner, SMART meeting, GP consult, local service.
- [ ] Schedule it.

Homework: Complete one support touchpoint this week.

Session 12 - Your 90-day operating manual

Objective: Lock in the system for the next 90 days.

Miss K teaching:

• The goal is not zero urges. The goal is urges no longer run the decision.

• You keep what you inspect: weekly scorecard + monthly review.

Do it now (write in the boxes):

My notes / answers

Action steps

- [ ] Write your top triggers, top rules, protocol cards, and early warning signs.
- [ ] Set your weekly review day.

Homework: Run the scorecard for 4 weeks and update one friction rule each month.

Templates

These pages are your toolkit. Print them. Screenshot them. Keep them on your phone. The goal is to make the safer choice the easier choice.

Trigger and urge log (7 days)

Date/time	Trigger	Urge 0-10	Story I noticed	What I did (10 min)	Outcome

Use one row per urge. The point is progress, not perfect tracking.

Trigger and urge log

Fill as you go. Small notes are enough.

Date/time	Trigger	Urge 0-10	Story I noticed	What I did (10 min)	Outcome

Trigger and urge log

Fill as you go. Small notes are enough.

Date/time	Trigger	Urge 0-10	Story I noticed	What I did (10 min)	Outcome

My ketamine loop map

Miss K instruction: map it like a system. No shame language. Just inputs and outputs.

Cues (what happens right before you want K):

Context (where/with who):

Access path (how it becomes available):

Story (the line your brain uses):

Immediate payoff (what it changes fast):

Aftermath cost (what it costs later):

First interruption point (earliest place you can break the chain):

My 72-hour safeguards

Pick the smallest set of actions that makes using harder immediately.

Friction moves I will run this week:

- [ ] Block/delete contacts linked to ketamine
- [ ] Remove kit/ritual items
- [ ] Remove stashes
- [ ] Avoid high-risk places for 30 days
- [ ] Money friction (reduce cash, spending limits, leave card at home if safe)
- [ ] Change route home / avoid pick-up points
- [ ] Pre-book travel home (no drift into afters)
- [ ] Accountability check-ins

My check-in person and time:

My danger window (time of day) and my off-ramp plan:

My 10-minute urge protocol (D-D-D-R)

Delay 10 minutes. Disrupt your body state. Defuse the story. Replace the job.

My disrupt action (choose 1-2):

My defuse sentence (one line):

My replacement action (10-minute version):

If the urge is still high after 10 minutes, I will:

- [ ] Repeat the timer
- [ ] Call/text my check-in person
- [ ] Leave the location
- [ ] Do my 60-minute replacement

Night out protocol

Miss K rule: decide before the night decides for you.

Before:

Arrival time	
Leave time	
Travel booked	
Buddy / check-in person	
Alcohol rule (if relevant)	
My 'leave now' warning sign	
Exit script	

After:

- [ ] Go straight to my minimum viable recovery routine
- [ ] Hydrate + eat something simple
- [ ] Set one small task for tomorrow
- [ ] Message my check-in person: 'Home safe'

Slip repair - 24-hour reset

A slip is data. Fix the system. Come back fast.

Where the relapse chain started (earliest point):

Which switch was on? (availability / proximity / cue / story / low resistance)

One friction move I will add immediately:

One support action I will take within 24 hours:

Reset checklist:

- [ ] Remove access again (contacts, kit, places)
- [ ] Sleep + food + water
- [ ] Short walk / shower to reset body state
- [ ] Write one lesson and one system change
- [ ] Contact a supporter

My 90-day operating manual

The goal is not zero urges. The goal is urges do not run the decision.

My top 3 triggers:

My top 3 rules (friction):

My early warning signs (drift):

My weekly review day and time:

Weekly scorecard:

Ketamine-free days (0-7)	
High-risk situations avoided	
Urge protocol used	
Average sleep (hours)	
Support touchpoints	
One improvement for next week	

Miss K sign-off: you are not broken. Your loop is. We fix loops.